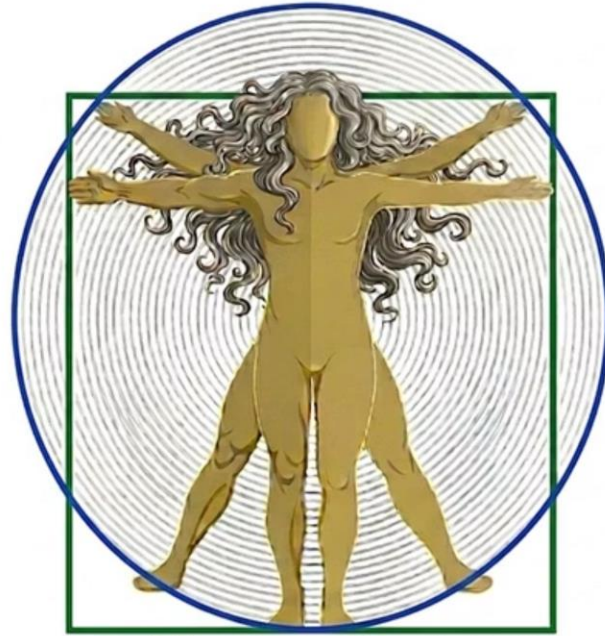
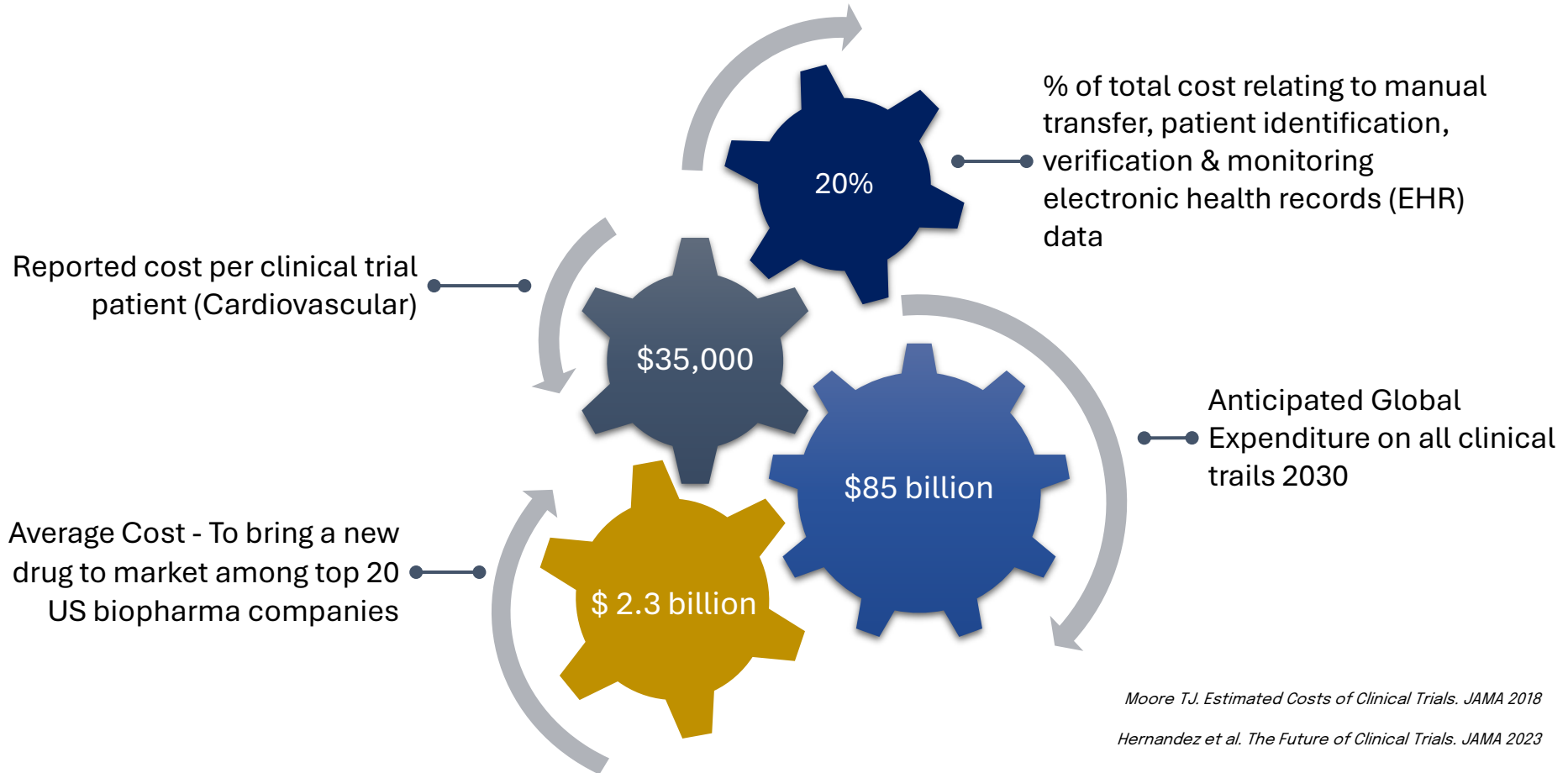




ClinTrialAI

Reimagining Clinical Trials with AI

Economics of Clinical Trials



The Problem



Patient recruitment

55%

Early Terminations: Cite poor patient recruitment as the primary cause

30%

Enrollment Costs: % of total trial costs attributed to patient enrollment alone

20%

Zero patients: % of activated trial sites that failed to enroll patients



Regulatory backlog



Data Management



Patient dropout

30%

Dropout rate after enrolment across all clinical trials

80%

Trials Delayed: % of clinical trials that fail to complete on time

Issues in Clinical Trials

The Compounding Cost of Delays

80%

Trials Delayed: % of clinical trials that fail to complete on time



Prolonged Operations

Extended trial timelines
inflate operational
expenditure



Delayed Patient Access to Therapies

Patients wait longer for
potentially life-saving
therapies



Patent Erosion

Effective patent life shrinks,
reducing commercial return

Why is Enrollment Tricky?

Patient supply is abundant but reliably finding and enrolling the right patients in a timely fashion is the rate limiting step



EHRs Built for Clinical Care Not Clinical Trials

EHRs are designed to capture clinical data and not to handle composite phenotypes, complex inclusion and exclusion criteria of clinical trials



Manual Chart Review

Trial Coordinators manually review thousands of charts in the EHR system, which is labour-intensive, slow and error-prone, with no viable scalability



Fragmented Workflows

Key trial steps (recruitment, screening, randomization, follow-up and closure) live in separate tools, causing handoff gaps and broken auditability

Clinical Trial Execution Phases and Gaps

Patient recruitment is the single most rate-limiting step in the entire trial lifecycle

Recruitment

Identify and attract potential participants for the study



Screening

Evaluate candidates to ensure they meet eligibility criteria.



Randomization

Assign participants to different groups by chance.



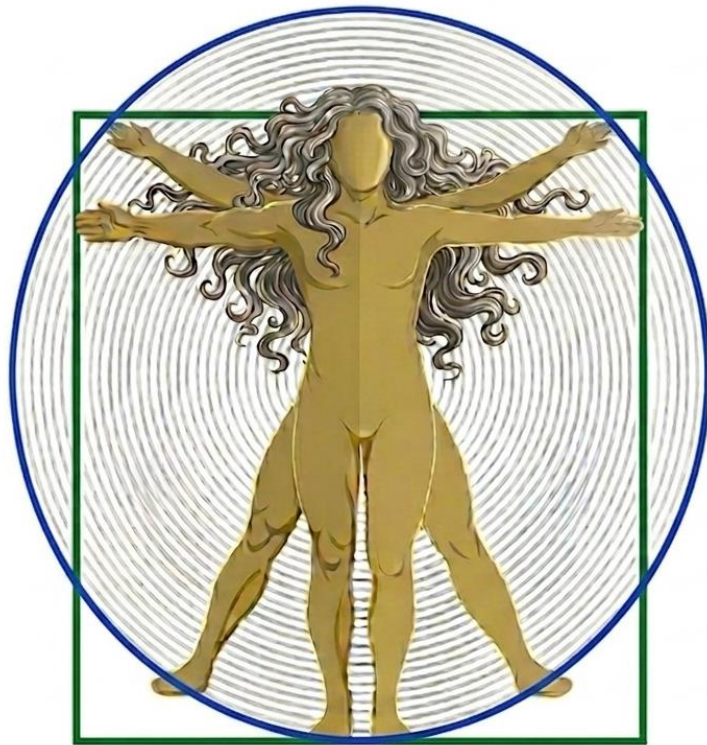
Follow-Up

Monitor participant progress and collect data over time



Patient dropout following randomization is another limitation

Introducing ClinTrialAI



What is ClinTrialAI?

ClinTrialAI is an end-to-end, AI-enabled clinical trial execution platform — integrating with EHRs while operating independently of them.

Recruit



Continuous real-time pre-screening and identification of eligible patients using complex, multi-dimensional criteria

Randomize



Built in protocol-driven randomization with full audit trail and compliance



Close

Streamlined data lock, regulatory submission, and trial close-out reporting

Screen

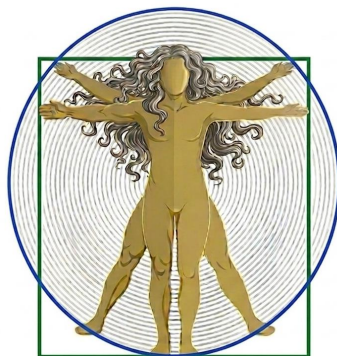


Automated eligibility verification across EHR data — labs, imaging, medications, and other unstructured data

Follow-Up



Longitudinal monitoring, automated scheduling, enhanced adherence, real-time data capture and automated uploading of CRFs



Platform Capabilities



AI Patient Identification

Multi-dimensional real time eligibility screening across EHR data — labs, imaging, medications and from other unstructured data (outpatient records, hospitalizations etc)



Automated Recruitment

Reduces reliance on manual coordinator labor, accelerating enrolment at scale



Operational Intelligence

Real-time visibility into site performance, enrolment rates, and trial health



Unified Trial Workflow

Screening, consent, randomization, follow-up, and reporting in one connected platform

Link to [modified trial process](#)

The Value Proposition

A Fundamental Reset of Trial Economics

ClinTrialAI converts the most unpredictable component of drug development into scalable, software-driven infrastructure

Improved Coordinator to Trials Ratio

1:7
or 8

Up from 1:5 — AI handles the scale

Improved Recruitment Speed

Hours

Patient identification reduced from 4-6 months to hours

Cost Drivers Addressed

7x

Across regulatory, staffing, data, IRB, and opportunity costs

Where the Savings Come From

Additional Recruitment

More patients per site drives revenue and accelerates completion

Coordinator Leverage

1:5 → 1:7 or 8 ratio reduces staffing costs

Regulatory Staff

Fewer amendments, renewals, and documentation months

Investigator Time

Reduced oversight burden

Data Management

Faster CRF completion and query resolution

IRB Continuing Review

Trials close before repeated annual renewals

Lower Opportunity Cost

Staff redeploy to new trials sooner

Why **ClinTrialAI** Over Others?



No other dominant product

- No dominant competitor exists in this space
- Opportunity to gain significant market share
- **ClinTrialAI** is addressing a gap in the clinical trials space



Truly End-to-End Platform

Unlike tools focused only on patient identification, **ClinTrialAI** covers the full trial lifecycle:

- AI-powered patient identification
- Automated trial invitations
- AI Retention Index — identifying patients at low risk of dropout post-randomization
- Screening automation
- Enhancing adherence
- Automated data uploads and CRF completion



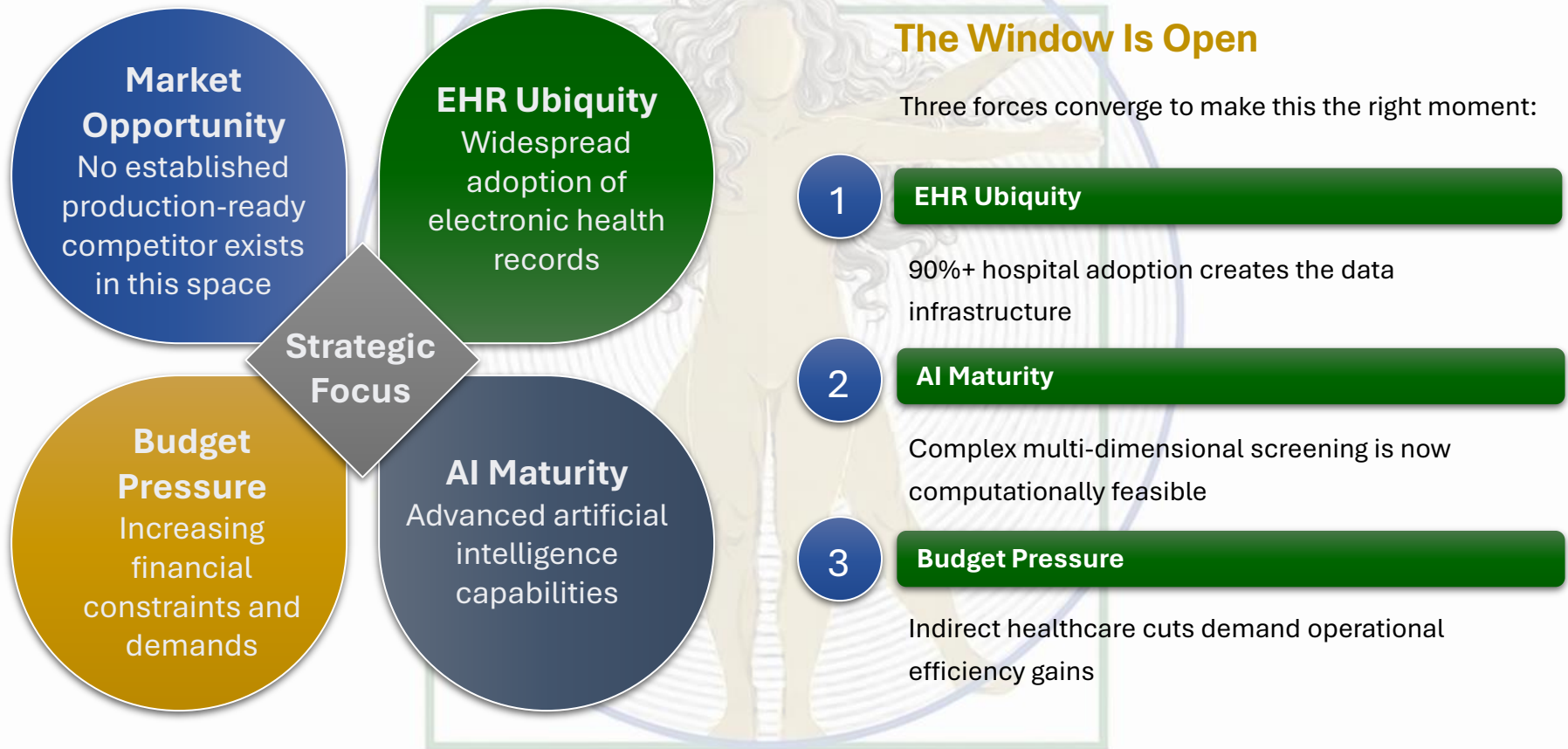
Lowest Operational Cost

Significantly lower cost than competing solutions proposed in the market due to off-shore engineering services built-in

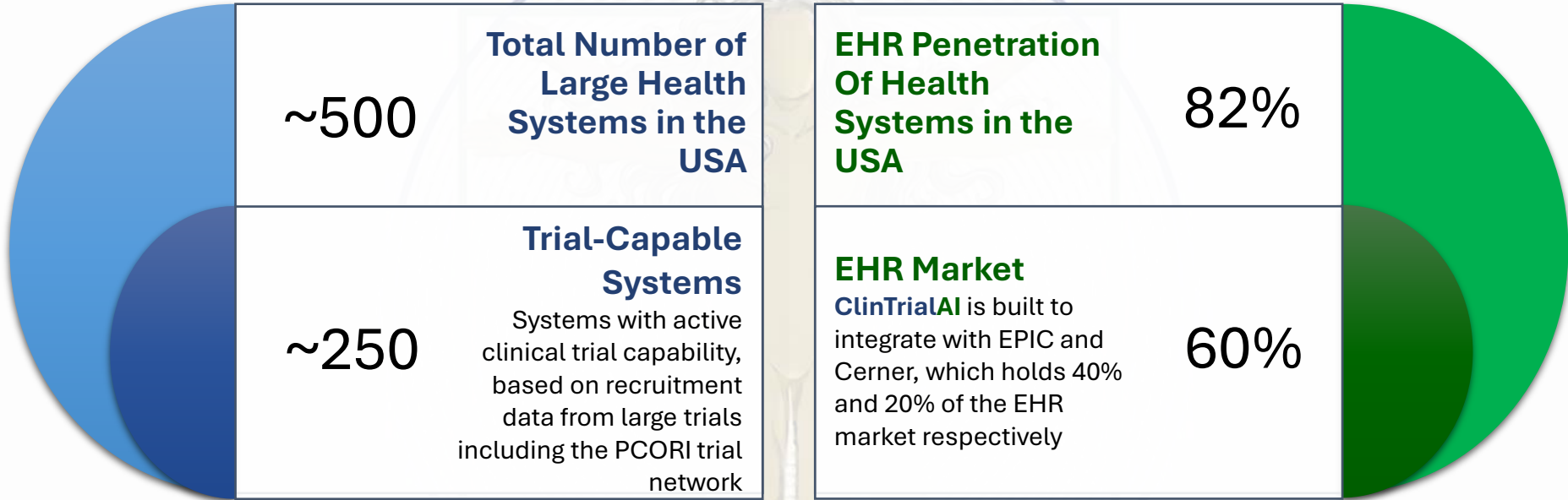


AI Retention Index: A proprietary **ClinTrialAI** tool predicting which enrolled patients are most likely to complete the trial, reducing dropout rates and protecting data integrity. Serves as a differentiator in the market

Why Now?



Total Addressable Market — USA



- Focusing on the EPIC and CERNER systems represents ClinTrialAI's immediate foothold market
- ClinTrialAI's initial target would be healthcare systems with clinical research capability, building a scalable B2B SaaS model with recurring revenue tied to trial volume.

Revenue Model

Subscription-Based SaaS Model

- **High Gross Profit margin ~80%**
- **Per trial cost decreases from \$40,000
→ \$20,000 as volume scales**
- **Engineer costs scale linearly with trial
volume**

Expenditure				
	Trials	3	5	10
Engineer		20,000	30,000	40,000
TOTAL		20,000	30,000	40,000

Revenue				
Number of Trials	3	5	10	
Gross Profit Margin	83%	80%	80%	
Total	120,000	150,000	200,000	
Per trial cost	40,000	30,000	20,000	

Leadership Team



Baskaran Selvaraj

CO-FOUNDER & CHIEF EXECUTIVE OFFICER

- CEO of Newdream Data Systems (> 100 employees)
- Deep expertise in enterprise data platforms, system integration, and scalable application solutions
- End-to-end data engineering, modern application development, secure high-performance systems



Dr. Varun Sundaram, MD, PhD, MSc (Oxon), FRCP(UK)

CO-FOUNDER & CHIEF SCIENTIFIC OFFICER

- Associate Professor of Medicine, Case Western Reserve University; Section Chief, Advanced Heart Failure, Louis Stokes Cleveland VA Medical Center, University Hospitals Cleveland Medical Center
- Heads Heart Failure Research Team including PhDs, PostDoc scholars, research fellows
- Expertise in Clinical Trials and Electronic Healthcare Records Research
- Author of 70+ publications in major journals including *Nature Medicine*, *Circulation*, *JACC*, *European Heart Journal*. NIH, VA & American Heart Association grant recipient



Dr. Chloe Bloom, MD, MSc, PhD

CHIEF MEDICAL OFFICER

- Senior Clinical Lecturer, Imperial College London; Honorary Respiratory Consultant
- Author of 70+ publications influencing major clinical guidelines and national policy
- Expertise in Electronic Health Records Research and Clinical Trials in the UK; national & international leadership in major respiratory societies